Mr. Surendra Mishra

Age / Sex : 25 YRS / M

Referred by : INDRA KIDNEY CARE & SURGICAL CENTER

Reg. no. : 7021

7021

Registered on : 22/10/2026 03:58 PM

Collected on : 22/10/2026 03:00 PM

Received on : 22/10/2026 03:01 PM

Reported on : 22/10/2026 03:58 PM

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## SEROLOGY & IMMUNOLOGY

### VIRAL MARKER

| TEST | VALUE | UNIT | REFERENCE |
|---|---|---|---|
| HIV (CARD TEST) | | | |
| HIV - 1 | NEGATIVE | | |
| HIV - 2 | NEGATIVE | | |
| **HEPATITIS C VIRUS, HCV** | **WEAKLY POSITIVE** | | |
| HBSAG | NEGATIVE | | |
| *Advice* | *RECOMEND HCV ELISA/ RNA FOR CONFIRMATION* | | |

**HbsAg** is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

**Anti HCV card test interpretation**

Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

**HIV Interpretation**

A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

## BIOCHEMISTRY

### KIDNEY FUNCTION TEST (KFT)

| TEST | | VALUE | UNIT | REFERENCE |
|---|---|---|---|---|
| RANDOM BLOOD SUGAR | | 101.6 | mg/dl | 70 - 140 |
| **SERUM UREA** | **H** | **118.2** | **mg/dl** | **19 - 45** |
| **SERUM CREATININE** | **H** | **16.27** | **mg/dl** | **0.72 - 1.43** |
| **SERUM PHOSPHORUS** | **H** | **8.15** | **mg/dl** | **2.5 - 4.5** |
| SERUM URIC ACID | | 7.18 | mg/dl | 3.5 - 7.2 |
| **SERUM SODIUM** | **L** | **134.1** | **mmol/L** | **136 - 146** |
| SERUM POTASSIUM | | 5.10 | mmol/L | 3.5 - 5.1 |
| ICALCIUM | | 1.20 | mmol/l | 1.13 - 1.33 |

| | |
|---|---|
| PH | 7.415 |

**Creatinine** is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

| Causes of Increased Serum Creatinine Level | Causes of Decreased Serum Creatinine Level |
|---|---|
| 1. Pre-renal, renal, and post-renal azotemia<br>2. Large amount of dietary meat<br>3. Active acromegaly and gigantism | 1. Pregnancy<br>2. Increasing age (reduction in muscle mass) |

~~~ End of report ~~~

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